Data sources and definitions

The bundled patient, pharmacy, and safety summary tables are synthetic teaching data created only to demonstrate a workflow. The task screenshot indicated supplied materials, but their links were not visible. Replace every demo CSV with the assigned, approved source files before reporting actual results.

A dispensed fill does not prove ingestion. Spontaneous adverse-event reports can be incomplete, duplicated, unverified, and affected by reporting behavior. Serious-report proportions describe submitted reports; they do not measure patient-level risk. Multiple products and events in a report do not establish a causal pair.

Privacy, bias, and quality

1. Use approved minimum-necessary, de-identified or aggregate data; restrict access.
2. Suppress small cells per organizational policy and avoid re-identification through filters.
3. Assess missingness and representation before subgroup comparisons.
4. Have clinical governance define review criteria; monitor false positives and workload.
5. Predefine the baseline, cohort, time window, and success thresholds.

Limitations

These examples are not supplied operational data. Results are illustrative and must not be used for medical care, medication changes, patient ranking, or product safety conclusions. A public adverse-event report signal cannot establish causation or incidence without additional validated evidence and appropriate study design.

Next steps

1. Obtain the assigned datasets and data dictionaries from the task materials.
2. Confirm lawful access, minimum-necessary fields, cohort rules, and refresh date.
3. Recompute metrics; validate with data owners and the pharmacy/clinical team.
4. Review privacy, bias, small-cell risks, and workload before a limited pilot.
5. Publish results with denominators, missingness, and uncertainty clearly shown.

Leadership ask

Approve a governed validation and pilot-design phase only; request real approved inputs before making operational decisions.

Source framework: FDA openFDA Drug Adverse Event reporting documentation. This project uses no downloaded patient records; public spontaneous-report counts still require careful, non-causal interpretation.